



MEMO
JABATAN FARMASI
HOSPITAL TUNKU AZIZAH
(Hospital Wanita & Kanak-Kanak)

Bil. (76) HWKKKL.100-1/4/9 JLD 3

Tarikh: 25 November 2025

Perkara	RUJUKAN UBAT-UBATAN BAGI WAD PERUBATAN PAEDIATRIK HOSPITAL TUNKU AZIZAH (VERSI NOVEMBER 2025)	
Kepada	Senarai Edaran	Salinan kepada: I. Pengarah II. Timbalan Pengarah I III. Timbalan Pengarah II
Daripada	Ketua Jabatan Farmasi	

YBrs. Dr./Tuan/Puan,

Saya dengan segala hormatnya merujuk perkara di atas.

2. Sukacitanya dimaklumkan bahawa Jabatan Farmasi Hospital Tunku Azizah (HTA) telah mengemaskini rujukan ubat-ubatan bagi Wad Perubatan Paediatric kepada **versi November 2025 (Lampiran A)**. Sehubungan itu, rujukan **versi Februari 2021** dan **Mac 2025** tidak lagi digunapakai.

3. Sebagai makluman tambahan, maklumat dos yang disertakan adalah panduan asas sahaja. Pihak YBrs. Dr./Tuan/Puan digalakkan merujuk sumber rujukan lain sekiranya memerlukan dos yang lebih terperinci mengikut indikasi. Ringkasan perubahan dos versi terkini boleh dirujuk pada **Lampiran B** dan **Lampiran C**.

4. Kerjasama dan perhatian daripada pihak YBrs. Dr./Tuan/Puan untuk memanjangkan makluman ini kepada anggota di bawah seliaan masing-masing adalah amat dihargai.

Sekian, terima kasih.

“MALAYSIA MADANI”

“BERKHIDMAT UNTUK NEGARA”

Saya yang menjalankan amanah,

(NAZARIAH BT HARON) RPh. 3593

Ketua Jabatan Farmasi,
Hospital Tunku Azizah

A

LJL/ain

✉ nurulainnadhrah@moh.gov.my

☎ 03-2600 3000 samb: 2706

SENARAI EDARAN

- i. **Dr. Sabeera Begum binti Kader Ibrahim**
Ketua Jabatan Perubatan Pediatrik
- ii. **Dr. Shahril bin Abu Bakar**
Ketua Jabatan Obstetrik Dan Ginekologi
- iii. **Dr. Bahruddin bin Saripudin**
Ketua Jabatan Pergigian Pediatrik
- iv. **Dr. Mohd Yusof bin Abdullah**
Ketua Jabatan Pembedahan Pediatrik
- v. **Dr. Thohiroh binti Abdul Razak**
Ketua Jabatan Anestesiologi Dan Rawatan Rapi
- vi. **Dr. Selvasingam Ratnasingam**
Ketua Jabatan Psikiatri Dan Kesihatan Mental
- vii. **Dr. Nor' Ashikin binti Johari**
Ketua Jabatan Ortopedik
- viii. **Dr. Yap Hsiao Ling**
Ketua Jabatan Kecemasan Pediatrik
- ix. **Dr. Soo Mun Yee**
Ketua Jabatan Otorinolaringologi
- x. **Dr. Norhafizah binti Hamzah**
Ketua Jabatan Oftalmologi
- xi. **Dr. Fadzlishah Johanabas bin Rosli**
Ketua Jabatan Pembedahan Neurologi
- xii. **Dr. Mohd Ali bin Mat Zain**
Ketua Jabatan Pembedahan Plastik Dan Rekonstruktif
- xiii. **Dr. Noor Ashani bin Md Yusoff**
Ketua Jabatan Urologi
- xiv. **Dr. Che Zubaidah binti Che Daud**
Ketua Jabatan Radiologi
- xv. **Encik Mohammad Nazli bin Mohamed Khalid**
Ketua Penyelia Hospital
- xvi. **KPJ Muslimah binti Yusof**
Ketua Unit Kejururawatan

PAEDIATRIC DOSES FOR ORAL MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

DRUG	PAEDIATRIC DOSE	STRENGTH
Anti-Tuberculosis	Isoniazid 10-15mg/kg/dose OD (max. 300mg)	Tab 100mg
	Rifampicin 10-20mg/kg/dose OD (max. 600mg)	Cap 150mg, Cap 300mg
	Ethambutol 15-25mg/kg/dose OD (max. 1000mg)	Tab 400mg,
	<u>Ethambutol 100 mg Dispersible Tablet:</u> Oral weight bands: 4-7 kg: 1 tab daily 8-11 kg: 2 tab daily 12-15 kg: 3 tab daily 16-24 kg: 4 tab daily ≥25 kg: use Film coated Tab	Dispersible Tab 100 mg
	Pyrazinamide 30-40mg/kg/dose OD (max. 2000mg)	Tab 500mg
	Pyridoxine 5-10mg/dose OD	Tab 10mg
Acyclovir	<u>Fixed Dose Combination (3FDC)</u> Rifampicin 75mg + Isoniazid 50mg + Pyrazinamide 150 mg Oral weight bands: 4-7 kg: 1 tab daily 8-11 kg: 2 tab daily 12-15 kg: 3 tab daily 16-24 kg: 4 tab daily ≥25 kg: Adult FDC tablet	Dispersible Tab Rifampicin 75mg + Isoniazid 50mg + Pyrazinamide 150 mg
	<u>Varicella (chicken pox)</u> 20mg/kg/dose QID (max. 800mg/dose) for 5 Days, within 24 hours of the rash. <u>Herpes zoster (shingles)</u> <12y/o: 20 mg/kg/dose QID (max. 800mg/dose) ≥12 y/o: 800 mg 5x/DAY <u>Herpes simplex (HSV)</u> 40–80 mg/kg/DAY, divided in 3–4 doses (max. 1g/DAY) for 5-10 Days	Syr 200mg/5ml, Tab 200mg, 800mg
Augmentin (Amoxicillin + Clavulanate)	<u>Syrup:</u> 20-25 mg/kg/dose BD (Max. 2g/DAY) <u>Tablet:</u> >25 kg: 625 mg TDS	Syr 228.5mg /5ml (7:1 ratio), Tab 625 mg (4:1 ratio)

DRUG	PAEDIATRIC DOSE	STRENGTH
Ampicillin	25mg/kg/dose QID (max. 500mg/dose, 2g/DAY) for 5-7 Days	Syr 125mg/5ml
Ampicillin + Sulbactam (Unasyn)	<30kg: 25-50mg/kg/DAY in 2 divided doses (max. 375mg BD) ≥30kg: 375-750mg BD	Syr 250mg/5ml, Tab 375mg
Amoxicillin	<u>Pneumonia</u> High dose: 80-90 mg/kg/DAY in 2-3 divided doses for 5-7 Days (max. 1g/dose) Experts recommend using at least 40mg/kg/dose BD to overcome resistance conferred by cell wall changes of the bacteria (pneumococcus) If unable to tolerate high dose (diarrhea), 45-50 mg/kg/DAY in 3 divided doses (max. 500mg/dose)	Syr 250mg/5ml, Cap 500mg
	<u>Tonsillitis/Pharyngitis</u> 50 mg/kg/DAY in 1-2 divided doses for 10 Days (max. 1g/DAY)	
	<u>Acute Otitis Media</u> 80-90 mg/kg/DAY in 2 divided doses for 7 days (max. 1g/dose)	
	<u>Other Indications, may consider</u> 45-90 mg/kg/DAY in 2-3 divided doses (max. 1g/dose)	
Azithromycin	<u>Atypical Infection</u> 10mg/kg (max 500mg) OD D1, then 5mg/kg (max 250 mg) OD D2-D5* OR 15mg/kg (max 500 mg) OD D1, then 7.5mg/kg (max 250 mg) OD D2-D5* *Duration may be longer according to paediatrician, case-by-case basis Other dosing regimen may be used, as per NAG or other references	Syr 200mg/5ml, Tab 250mg
	<u>Pertussis</u> 1-5 mo: 10mg/kg OD for 5/7 ≥6 mo: 10mg/kg OD D1 (max 500mg), then 5mg/kg OD D2-D5 (max 250 mg)	

PAEDIATRIC DOSES FOR ORAL MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

DRUG	PAEDIATRIC DOSE	STRENGTH
Cefuroxime	15mg/kg/dose BD (max. 500mg BD)	Syr 125mg/5ml, Tab 250 mg
Cephalexin	12.5-25 mg/kg/dose BD (max. 2g/DAY) Cellulitis for 5-7 Days UTI for 7-10 Days Tonsillopharyngitis for 10 Days <u>UTI Prophylaxis</u> 12.5 mg/kg/dose ON (max. 125 mg/dose)	Syr 125mg/5ml, Cap 250 mg
Cetirizine	6 mo-1yr: 2.5 mg OD >1yr-5yr: 2.5- 5 mg OD ≥6 yr: 5-10 mg OD	Tab 10 mg
Chlorpheniramine	≥2 y/o: 0.1 mg/kg/dose TDS (Adult 4mg TDS)	Syr 2mg/5ml, Tab 4 mg
Ciprofloxacin	10-20 mg/kg/dose BD (max. 500mg BD)	Tab 250mg
Clarithromycin	7.5 - 15 mg/kg/dose BD (max. 500 mg BD)	Syr 125mg/5ml, Tab 250mg
Cloxacillin	<u>Mild infection</u> <25kg: 15-25 mg/kg/dose QID (max. 1g/DAY) ≥25kg: 500 mg QID *Use IV for severe infection	Syr 125mg/5ml, Cap 250 mg
Domperidone	0.2-0.5mg/kg/dose (adult 10-20 mg) TDS-QID	Syr 1mg/ml, Tab 10mg
Erythromycin Ethylsuccinate	20-25 mg/kg/dose BD (Adult 800 mg BD) Tonsillopharyngitis for 10 Days <u>Gut prokinetic</u> 2mg/kg/dose TDS	Syr 200mg/5ml, Tab 400mg
Folic Acid	< 4 yr: 0.1-0.25mg OD ≥ 4 yr: 0.5-1mg OD	*Syr 1mg/ml (stable 60 Days), Tab 5mg
Ibuprofen	5-10 mg/kg/dose TDS-QID (max. 40 mg/kg/DAY, 2.4g/DAY)	Tab 200 mg

DRUG	PAEDIATRIC DOSE	STRENGTH
Loratadine	1yr-<12kg: 2.5 mg OD 12kg-30kg: 5 mg OD >30kg: 10 mg OD	Syr 1mg/ml, Tab 10 mg
Maltofer	Prophylaxis: elemental iron 2mg/kg/DAY OD Treatment: 6mg/kg/DAY OD (Adult 100 mg-300 mg daily)	Syr containing Elemental iron 10mg/ml
Metronidazole	>4 weeks: 15mg/kg/dose (max: 1g) STAT, then 7.5mg/kg/dose TDS <u>C.Difficile</u> 10 mg/kg/dose TDS for 10 Days (max. 500 mg/dose)	Syr 200mg/5ml, Tab 200mg
Multivitamin (MVT)	<u>Syrup MVT</u> 1-3 y/o: 1 ml OD 4-12 y/o: 2.5 ml OD <u>Appeton Baby Drops</u> <1y/o: 1 ml OD >1y/o: Gastro clinic Biliary atresia cases (for all ages) Max: 2ml	Vit. composition per ml: <u>Syr MVT (Uphavit)</u> B1 0.5mg, B2 0.25mg, B3 2mg, B6 0.2mg, B12 0.5mcg, B5 0.05mg, A 600iu, D 80iu <u>Appeton baby drops</u> B2 0.6mg, B3 8mg, B6 0.4mg, A 1500iu, D 400iu, E 5iu, lysine 15mg, Taurine 15mg
Omeprazole	0.4-0.8mg/kg/dose OD-BD (Adult 20-40 mg/dose)	*Syr 2mg/ml (stable 30 Days), Cap 20mg
Oseltamivir	<9 mo: 3 mg/kg/dose BD 9-11 mo: 3.5 mg/kg/dose BD <u>1-12 years old:</u> ≤15 kg: 30mg BD >15-23kg: 45mg BD >23-40kg: 60mg BD >40 kg: 75mg BD Treatment for 5 Days Paediatrician may increase one dose higher/extend duration in severe disease	Syr 60mg/5ml, cap 75mg
Prednisolone	Asthma: 1-2 mg/kg/dose OD (max. 40 mg)	Tab 5mg

PAEDIATRIC DOSES FOR ORAL MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

DRUG	PAEDIATRIC DOSE	STRENGTH
Paracetamol	<u>Oral</u> 15mg/kg/dose q6H-q4H OR 10mg/kg/dose (patient with hepatitis) Max. 4 doses in 24 hours Max. 4g/DAY in children <u>Suppository</u> 10-20mg/kg/dose q6H-q4H May give loading dose: 25-40 mg/kg STAT (Max 1g)	Syr 250mg/5ml, Tab 500 mg, Supp 125 mg, Supp 250 mg
Penicillin V	15 mg/kg/dose QID (max. 500mg QID) <u>Prophylaxis (Nephrotic Syndrome)</u> 125 mg BD (1-5 years age), 250 mg BD (6-12 years), 500 mg BD (> 12 years)	Syr 125mg/5ml, Cap 125mg
Sulpha-methoxazole + Trimethoprim (Bactrim)	Dose based on TMP component 4-6 mg/kg/dose BD (max. 160mg BD) <u>UTI prophylaxis</u> 2 mg/kg/dose (TMP dose) ON <u>PCP Prophylaxis</u> 75 mg/m ² (TMP dose) BD on 3 consecutive Days	Syr 200mg+40mg (TMP)/5ml, Tab 400mg + 80mg (TMP)

Remarks:

1. Doses stated above are for general reference only. Please refer to other references for indication-specific doses.
2. Dose may be prescribed +/- 10% of the recommended range to the nearest whole number for ease of administration.

References:

1. National Antimicrobial Guideline (NAG), 4th Edition 2024
2. Micromedex Paediatric Reference database 2022
3. CPG Management of Tuberculosis, 4th Edition 2021
4. Paediatric Protocol for Malaysian Hospitals, 4th Edition

PAEDIATRIC DOSES FOR ORAL MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

Prepared by:



Nurul Ain Nadhrah
binti Muhamed Edil
Pharmacist
Hospital Tunku Azizah

Reviewed by:



Dr. Nazedah
binti Ain @ Ibrahim
Pharmacist
Hospital Tunku Azizah

Approved by:



Pn. Nazariah binti
Haron
Head of Pharmacy
Department
Hospital Tunku Azizah

Approved by:



Dr. Norshireen Nazli
binti Abdul Razak
Consultant General
Paediatrics
Hospital Tunku Azizah

PAEDIATRIC DOSES FOR INJECTION MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

DRUG	PAEDIATRIC DOSE	STRENGTH
IV Acyclovir	<u>Herpes simplex encephalitis</u> 1-4 mo: 20mg/kg/dose TDS >4mo-12 y/o: 10-15 mg/kg/dose TDS	250mg/vial
	<u>Chicken Pox (varicella zoster) & Shingles (herpes zoster)</u> 10mg/kg/dose TDS	
IV Augmentin (Amoxicillin + Clavulanate)	0-3 mo: 30 mg/kg/dose BD >3 mo: 30 mg/kg/dose TDS (max. 1.2g/dose)	600mg/vial, 1.2g/vial ratio 5:1
IV Ampicillin	25-50mg/kg/dose QID (max.2g/dose) <u>Severe infection (meningitis, endocarditis)</u> 200 to 300 mg/kg/DAY divided every 4-6H; max. 12g/DAY	500mg/vial
IV Azithromycin	<u>Standard Regime</u> 10 mg/kg/dose OD D1 (max. 500mg/dose); then 5 mg/kg/dose OD D2-D5 (max. 250mg/dose) Refer NAG for other dosing regimen	500mg/vial
IV Ampicillin + Sulbactam (Unasyn, 2:1 ratio)	Dose based on Ampicillin component <u>Mild/moderate infection:</u> 100-200mg/kg/DAY in 4 divided doses <u>Severe infection:</u> 200-300mg/kg/DAY in 4 divided doses (max 1g/dose of Sulbactam)	1.5g/vial (1g Ampicillin + 500mg Sulbactam)
IV Benzylpenicillin (1mg = 1667u)	<u>Tonsilopharyngitis</u> 25,000u/kg/dose QID <u>Pneumonia</u> 50,000u/kg/dose QID <u>Meningitis</u> 100,000u/kg/dose QID <u>Presumed sepsis (< 3 mo)</u> 100,000u/kg/dose BD (max. 4MU/dose or 24MU/DAY)	1MU/vial 5MU/vial

DRUG	PAEDIATRIC DOSE	STRENGTH
IV Cefepime	50 mg/kg/dose BD or TDS (max. 2g/dose, 6g/DAY)	1g/vial (ARF)
IV Cefotaxime	50mg/kg/dose TDS or QID <u>Meningitis</u> 50-75mg/kg/dose QID (max. 2g/dose, 12g/DAY)	1g/vial
IV Ceftazidime	50 mg/kg/dose TDS-QID (max. 2g/dose, 9g per DAY)	1g/vial 2g /vial
IV Ceftriaxone	>3 months: 50-75 mg/kg/dose OD (max. 2g/dose) <u>Meningitis</u> 50 mg/kg/dose BD (max. 2g/dose)	1g/vial
IV Cefuroxime	100 mg/kg/DAY in 3 divided doses <u>Severe infection</u> 50mg/kg/dose TDS (max. 2g/dose, 6g/DAY)	750mg/vial 1.5g/vial
IV Cloxacillin	200-300 mg/kg/DAY in 4-6 divided doses (max. 2g/dose, 12g/DAY)	250mg/vial 500mg/vial
IV Flumazenil (for benzo reversal)	5 mcg/kg/dose (max. 0.2mg, 2ml) over 15-30 sec (not > 0.2mg/min) Repeat dose every 60s until awake (max. total dose 50mcg/kg, adult 2mg)	0.1mg/ml = 100mcg/ml (5 ml per vial)
IV Frusemide	0.5-1 mg/kg/dose OD-QID	10mg/vial
IV Gentamicin	5 mg/kg/dose OD TDM on 3 rd dose Pre sampling: 30 min or just before next dose Post: 1H after dose started Pre level: <1.00 mg/L Post level: 5 – 20 mg/L	80mg/2ml per vial
IV Hydrocortisone	<u>Acute Asthma</u> 4 mg/kg/dose QID (max. 100mg/dose)	100mg/vial

PAEDIATRIC DOSES FOR INJECTION MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

DRUG	PAEDIATRIC DOSE	STRENGTH
IV Methylprednisolone	<u>Anti-inflammatory:</u> 1- 2 mg/kg/DAY in divided doses q6-12H <u>Pulse therapy:</u> 30mg/kg/DAY OD or in divided doses (max. 1g/DAY)	500mg/vial
IV Metronidazole	15 mg/kg STAT, then 7.5 mg/kg/dose TDS (max. 750 mg/dose)	500mg/vial
IV Midazolam	0.1-0.2 mg/kg/dose (max. 0.5mg/kg, max. dose 5mg) Antidote (Flumazenil)	5mg/ml (1ml and 3ml vial)
IV Morphine	0.1-0.2 mg/kg/dose (Max. 5 mg/dose) Antidote (Naloxone)	10mg/ml per vial
IV Naloxone	0.01mg/kg/dose (adult 0.4mg) every 2 min x 4 if required	0.4mg/ml per vial
IV Omeprazole	1mg/kg/dose BD (max. 40mg/dose)	40mg/vial
IV Pethidine	0.5-1mg/kg/dose (max.25-50mg)	50mg/ml (1ml and 2ml vial)
IV Phenytoin	<u>Loading</u> 15-20 mg/kg/dose (max. 1.25g) over 30 min (max rate 50 mg/min) with cardiac monitoring Dilute in NS, max concentration 10mg/ml <u>Maintenance:</u> 5mg/kg/DAY in 2 divided doses	250mg/5ml per vial
IV Phenobarbitone	<u>Loading</u> 20mg/kg/dose (max. 1g) over 1H (max rate 30 mg/min) with BP monitoring. Dilute in NS to 10- 20mg/ml. <u>Maintenance:</u> 5mg/kg/DAY in 1-2 divided doses	200mg/ml per vial

DRUG	PAEDIATRIC DOSE	STRENGTH
IV Piperacillin + Tazobactam	110mg/kg/dose TDS-QID (Max. 4.5g/dose)	4.5g/vial (4g piperacillin) 8:1 ratio (ARF)
IV Promethazine	0.5-1.5mg/kg/dose (max. 25-100mg)	50mg/2ml per vial
IV Bactrim (Sulpha-methoxazole + Trimethoprim)	Dose based on TMP component <u>PCP/ Stenotrophomonas treatment</u> 15-20mg/kg/DAY in 3-4 divided doses (max. 960 mg/DAY of trimethoprim) <u>PCP prophylaxis</u> 5mg/kg/dose in 2 divided doses 3 x per week (max. 160mg/dose)	Sulpha-methoxazole 400mg + TMP 80mg per vial
IV Vancomycin	15 mg/kg/dose 6H (max. 2g/DAY unless unable to achieve therapeutic level) <u>TDM after 24 hours</u> Pre sampling: 30 min or just before next dose Post: 1H after completion of infusion (e.g. infusion over 2H, post sampling taken at 3H after the start of infusion) Pre range: 10 – 20 mg/L Post range: 20 – 40 mg/L AUC: 400-600mg.hr/L	500mg/vial (ARF)

Remarks:

- Doses stated above are for general reference only. Please refer to other references for indication-specific doses.
- ARF- Antimicrobial Request Form (pink form).

References:

- National Antimicrobial Guideline (NAG), 4th Edition 2024
- Micromedex Paediatric Reference database 2022
- CPG Management of Tuberculosis, 4th Edition 2021
- Paediatric Protocol for Malaysian Hospitals, 4th Edition 2019
- Frank Shann, 16th Edition 2017

PAEDIATRIC DOSES FOR INJECTION MEDICATION GENERAL WARD, HOSPITAL TUNKU AZIZAH (WCHKL)

Prepared by:

Nurul Ain Nadhrah
binti Muhamed Edil
Pharmacist
Hospital Tunku Azizah

Reviewed by:

Dr. Nazedah
binti Ain @ Ibrahim
Pharmacist
Hospital Tunku Azizah

Approved by:

Pn. Nazariah binti
Haron
Head of Pharmacy
Department
Hospital Tunku Azizah

Approved by:

Dr. Norshireen Nazli
binti Abdul Razak
Consultant General
Paediatrics
Hospital Tunku Azizah

Ringkasan Perubahan Dos Bagi Sediaan Oral

Ubat	Dos Versi Februari 2021	Dos Baharu Versi November 2025
PO Acyclovir	<u>Herpes Zoster (shingles)</u> >12 y/o 800 mg 5x/day	<u>Herpes Zoster (shingles)</u> <12y/o: 20 mg/kg/dose QID (max. 800mg/dose) >12 y/o: 800 mg 5x/DAY
PO Amoxicillin	25-30mg/kg TDS	<u>Pneumonia</u> High dose: 80-90 mg/kg/DAY in 2-3 divided doses for 5-7 Days (max. 1g/dose) Experts recommend using at least 40mg/kg/dose BD to overcome resistance conferred by cell wall changes of the bacteria (pneumococcus) If unable to tolerate high dose (diarrhea), 45-50 mg/kg/DAY in 3 divided doses (max. 500mg/dose) <u>Tonsillitis/Pharyngitis</u> 50 mg/kg/DAY in 1-2 divided doses for 10 Days(max. 1g/DAY) <u>Acute Otitis Media</u> 80-90 mg/kg/DAY in 2 divided doses for 7 days (max. 1g/dose) <u>Other Indications, may consider</u> 45-90 mg/kg/DAY in 2-3 divided doses (max. 1g/dose)
PO Augmentin (Amoxicillin + Clavulanate)	Syrup: 20-30 mg/kg BD Tablet: 625 mg TDS	Syrup: 20-25 mg/kg/dose BD (Max. 2g/DAY) Tablet: >25 kg: 625 mg TDS
PO Azithromycin	<u>Atypical infection</u> 15mg/kg (max:500mg) D1 then 7.5 mg/kg (max:250mg) D2-D5 OR 15 mg/kg (max:500mg) OD for 3/7	<u>Atypical Infection</u> 10mg/kg (max 500mg) OD D1, then 5mg/kg (max 250 mg) OD D2-D5* OR 15mg/kg (max 500 mg) OD D1, then 7.5mg/kg (max 250 mg) OD D2-D5* *Duration may be longer according to paediatrician, case-by-case basis

Ubat	Dos Versi Februari 2021	Dos Baharu Versi November 2025
PO Cephalexin	<u>Skin/UTI</u> 12.5mg/kg (max 500mg) QID OR 25 mg/kg (max:500mg) BD <u>UTI prophylaxis</u> 5mg/kg ON	12.5-25 mg/kg/dose BD (max. 2g/DAY) Cellulitis for 5-7 Days UTI for 7-10 Days Tonsillopharyngitis for 10 Days <u>UTI Prophylaxis</u> 12.5 mg/kg/dose ON (max. 125 mg/dose)
PO Cloxacillin	15 mg/kg QID (max: 1g QID)	<u>Mild infection</u> <25kg: 15-25 mg/kg/dose QID (max. 1g/DAY) ≥25kg: 500 mg QID *Use IV for severe infection
PO Erythromycin	20 mg/kg BD (max 400 mg BD)	20-25 mg/kg/dose BD (Adult 800 mg BD)
PO MVT	1-3 y/o: 1ml OD 4-12 y/o: 2.5 ml OD	<u>Syrup MVT</u> 1-3 y/o: 1 ml OD 4-12 y/o: 2.5 ml OD <u>Appeton Baby Drops</u> <1y/o: 1 ml OD >1y/o: Gastro clinic Biliary atresia cases (for all ages) Max: 2ml
PO Oseltamivir	<3 mo: 12mg BD 3-5 mo: 20 mg BD 6-11 mo: 25 mg BD 1-12 years old: ≤15 kg: 30mg BD >15-23kg: 45mg BD >23-40kg: 60mg BD >40 kg: 75mg BD	<9 mo: 3 mg/kg/dose BD 9-11 mo: 3.5 mg/kg/dose BD 1-12 years old: ≤15 kg: 30mg BD >15-23kg: 45mg BD >23-40kg: 60mg BD >40 kg: 75mg BD

Ringkasan Perubahan Dos Bagi Sediaan Injeksi

Ubat	Dos Versi Februari 2021	Dos Baharu Versi November 2025
IV Acyclovir	<u>Herpes simplex encephalitis</u> 20 mg/kg/dose TDS (max 60 mg/kg/day)	<u>Herpes simplex encephalitis</u> 1-4 mo: 20mg/kg/dose TDS >4mo-12 y/o: 10-15 mg/kg/dose TDS
IV Augmentin	30 mg/kg/dose TDS (max. 1.2g/dose)	0-3 mo: 30 mg/kg/dose BD >3 mo: 30 mg/kg/dose TDS (max. 1.2g/dose)
IV Azithromycin	15 mg/kg/ Day 1 (max. 500mg); then 5 mg/kg/day (max. 250mg)	<u>Standard Regime</u> 10 mg/kg/dose OD D1 (max. 500mg/dose); then 5 mg/kg/dose OD D2-D5 (max. 250mg/dose) Refer NAG for other dosing regimen
IV Metronidazole	15 mg/kg STAT, then 7.5 mg/kg/dose TDS (max. 500 mg/dose)	15 mg/kg STAT, then 7.5 mg/kg/dose TDS (max. 750 mg/dose)